

Prediction of Chronic Disease Risk Using Nutrition, Lifestyle & Clinical Data from The Irish Longitudinal Study on Ageing (TILDA)



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Abstract

Background: Chronic diseases such as cardiovascular disease, diabetes, frailty, and obesity represent major contributors to morbidity in older adults. While traditional prediction models focus heavily on clinical and functional indicators, the contribution of nutritional and lifestyle factors to early risk detection remains underexplored.

Methods: This thesis applies machine learning techniques to data from TILDA, a national representative longitudinal study of adults aged 50+. Five waves of socio-demographic, clinical, nutritional, psychological, and behavioural data, along with the externally produced RAND Harmonised TILDA dataset, were systematically cleaned and standardised using a multi stage preprocessing pipeline. Predictive models, including (*mention models used*) were developed to estimate the risk of chronic disease and frailty related outcomes. Model performance was evaluated using (*mention examples*), and feature importance was used to assess predictive value.

Results: TBD

Conclusions: TBD

Keywords: ageing, (AI), ML, predictive modelling, frailty, chronic disease, nutrition, TILDA

Declaration

I Abderahman Haouit declare that I have produced this thesis without the prohibited assistance of third parties and without making use of aids other than those specified; notions taken over directly or indirectly from other sources have been identified as such. This thesis has not previously been presented in identical or similar form to any other Irish or foreign examination board.

The thesis work was conducted from 2025 to 2026 under the supervision of Dr. Alison O'Connor at University of Limerick.

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Your dedication goes here. Open the dedication.tex file found at:
0_frontmatter/dedication.tex

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List of Abbreviations

ADL	Activities of Daily Living
AI	Artificial Intelligence
AUC	Area Under the ROC Curve
AUROC	Area Under the Receiver Operating Characteristic Curve
BMI	Body Mass Index
BP	Blood Pressure from Sphygmomanometer
brainPAD	Brain predicted age difference
CES-D	Center for Epidemiologic Studies Depression Scale
COG	Scales and Questions on Cognition
CRT	Choice Reaction Time (Cognitive Test)
CV	Cross-Validation
DIS	Disability, Functional Impairment and Helpers
DNN	Deep Neural Network
ELSA	The English Longitudinal Study of Aging
EU	European Union
FOF	Fear of Falling
FR	Frailty, Gait, Exercise, Falls and Fractures
GBM	Gradient Boosting Machine
GDPR	General Data Protection Regulation
HADS	Hospital Anxiety and Depression Scale
HRS	The Health and Retirement Study, (a longitudinal study of ageing in the United States)

List of Abbreviations

IDS	The Intellectual Disability Supplement
INC	Income Variables
ISSDA	The Irish Social Science Data Archive
MD	Medication Data
MGS	Maximum Gait Speed
ML	Machine Learning
MVPA	Moderate-to-Vigorous Physical Activity
NOS	Newcastle-Ottawa Scale
OR	Odds Ratio
PA	Physical Activity
RF	Random Forest
SCR	Screening Tests
SHAP	Shapley Additive Explanations
SHARE	The Survey of Health, Aging and Retirement in Europe
SLR	Systematic Literature Review
SOC	Social Variables
SR	Systematic Review
TILDA	The Irish Longitudinal Study on Ageing
TUG	Timed Up and Go (mobility test)
UGS	Usual Gait Speed
WHO	World Health Organization

List of Abbreviations

Chapter 1

Introduction

Chronic conditions including cardiovascular disease, diabetes, and frailty are leading contributors to morbidity and mortality in ageing populations [4, 5]. These conditions are associated with declines in functional capacity (e.g., gait speed, grip strength [6]), increased healthcare utilisation [7], and reduced quality of life [8].

The financial and societal burden is substantial. In Europe, poor diet is a major risk factor for non communicable diseases, contributing billions in healthcare costs annually [9]. In Ireland, malnutrition alone is estimated to cost over €1.4 billion per year [10]. Recognising this, international and national policies including the World Health Organization (WHO) Global Action Plan on Noncommunicable Diseases and Ireland’s *Healthy Ireland* framework highlights nutrition and lifestyle interventions as central to healthcare strategy.

In parallel, (Artificial Intelligence (AI)) and (Machine Learning (ML)) are increasingly applied in healthcare to support risk prediction and personalised intervention. However, European regulations such as the General Data Protection Regulation (GDPR) and the forthcoming European Union (EU) AI Act impose strict requirements for transparency, explainability, and governance. This regulatory context underscores the importance of interpretable and ethically robust ML approaches.

Traditional risk prediction models have largely relied on clinical biomarkers such as blood pressure and lipid profiles. Yet, growing evidence shows that broader predictors including nutrition [11], physical activity [12], mental health [13],

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and social determinants [14] improve predictive performance. Estimating the likelihood of developing a condition given certain risk factors, help quantify the influence of these predictors in clinical decision making.

(The Irish Longitudinal Study on Ageing (TILDA))¹ provides a nationally representative cohort of adults aged 50+ in Ireland, surveyed across multiple domains in repeated waves (waves 1-5). The raw wave datasets capture the full richness of TILDA specific measures relevant to Irish health, policy, and society. In parallel, the Harmonised TILDA dataset restructures selected variables to align with the RAND (The Health and Retirement Study, (a longitudinal study of ageing in the United States) (HRS)) framework, enabling cross national comparison with equivalent studies such as HRS (US), The English Longitudinal Study of Aging (ELSA) (UK), The Survey of Health, Aging and Retirement in Europe (SHARE) (continental Europe).

This harmonisation enhances international comparability, though at the cost of excluding some study specific variables. An additional dataset, the The Intellectual Disability Supplement (IDS), extends coverage to individuals with disabilities, improving inclusivity and reducing sampling bias. Together, these resources offer both detailed national data and a platform for internationally comparable, generalisable analyses.

Despite these advantages, existing ML research on chronic disease prediction faces persistent limitations: over reliance on clinical predictors, limited incorporation of lifestyle and social variables, predominantly cross sectional study designs that fail to capture trajectories, and a lack of interpretability consistent with emerging AI governance standards.

This thesis addresses these gaps by integrating TILDA’s longitudinal, multidomain data with interpretable ML methods, demonstrating how nutrition, physical activity, and mental health can be combined with traditional clinical markers to improve prediction of ageing related outcomes.

¹TILDA data are pseudonymised and accessed under licence via The Irish Social Science Data Archive (ISSDA). Use adheres to UL S&E ethics approval and data governance requirements.

1.1 Research Problem and Motivation

Despite increasing policy interest in lifestyle and nutrition and advances in ML methods, approaches to chronic disease prediction in older adults remain limited in several ways.

- **Limited Integration of Multidimensional Data:** Most models prioritise clinical variables while underutilising modifiable lifestyle and nutrition factors [11, 15].
- **Underuse of Longitudinal Dynamics:** Few studies leverage repeated measures to capture developments of risk, such as frailty progression [16]. Probabilistic reasoning can help estimate the likelihood of developing a condition at a future time given earlier measurements, for example calculating $P(\text{Frailty at wave 5} \mid \text{Gait speed at wave 1, Nutrition at wave 2})$.

This approach helps quantify how early indicators influence later outcomes and supports better feature selection and timing of interventions in predictive models.

- **Lack of Explainability:** Many ML models are not interpretable, limiting clinical adoption and failing to meet emerging regulatory requirements for trustworthy AI in healthcare [17].

Addressing these challenges is essential to improve predictive accuracy, support evidence based interventions, and ensure alignment with evolving policy and regulatory priorities.

1.2 Research Aims and Objectives

The goal of this thesis is to develop and validate ML models that integrate nutrition, lifestyle, and mental health features with clinical data to improve prediction of chronic disease and ageing related outcomes in a representative cohort of older adults. This goal will be achieved through the following objectives:

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- Review existing applications of ML for chronic disease prediction in ageing populations, with emphasis on gaps in lifestyle and nutrition integration.
- Preprocess and harmonise TILDA waves 1-5, and engineer multidomain features (nutrition, activity, mobility, mental health, clinical indicators).
- Develop and compare baseline statistical models (e.g., logistic regression) with advanced interpretable ML methods (Random Forest (RF), Gradient Boosting Machine (GBM)).
- Evaluate models using discrimination (Area Under the ROC Curve (AUC)), calibration, and error metrics (accuracy, precision, recall, F1-score), perform ablation analyses to assess the incremental value of lifestyle and nutrition predictors.
- Identify actionable predictors (e.g., vitamin D status, gait speed reserve) and highlight modifiable risk factors for intervention.
- Document ethical, governance, and reproducibility practices for handling longitudinal pseudonymised health data.

1.3 Contributions of the Research

This research makes contributions at three levels:

- **Empirical:** Demonstrates how nutrition, lifestyle, and mental health features enhance prediction when combined with established clinical baselines.
- **Methodological:** Provides a reproducible pipeline for harmonising longitudinal TILDA data and integrating multidomain features into ML workflows.
- **Clinical:** Identifies modifiable predictors with relevance for targeted interventions and policy to promote healthy ageing.

1.4 Thesis Outline

In *Chapter 2* a literature review on ML prediction of chronic disease in ageing populations, with emphasis on lifestyle and nutrition predictors and methodological considerations is presented. *Chapter 3* discusses the TILDA study design, data preprocessing, harmonisation, and feature engineering (Flow Charts...). In *Chapter 4* the modelling framework is outlined. This includes baseline and advanced ML, hyperparameter tuning, validation, and evaluation metrics. *Chapter 5* shows the model result, including model comparisons, assessments of feature importance, and an ablation analyses. In *Chapter 6* the implications, limitations, ethical considerations are discussed, and future directions for the research proposed. Additional supporting documents are included in appendices.

Chapter 2

Literature Review

This chapter reviews literature on ageing, chronic disease, and predictive modelling using TILDA. The focus is on three domains central to later life health: frailty and physical function, mental health, and nutrition and biological ageing. Chronic conditions including multimorbidity, frailty, and mental health difficulties are leading causes of morbidity and reduced quality of life in older adults, often overlapping to accelerate functional decline. In line with the WHO healthy ageing framework, research using TILDA highlights frailty, mental health, and nutrition as interconnected determinants of later life outcomes. The following subsections examine each domain in turn.

2.1 Methodology

Several sources describe how systematic and integrative reviews are commonly carried out in fields such as software engineering, health, and education. The approach followed key principles for systematic mapping, for example defining research questions, identifying relevant studies through structured database searches, applying inclusion and exclusion criteria, and categorising results as outlined in [1]. The integrative literature review [18] helped structure and connect findings across studies, while [19, 20] guided the organisation of ideas. In related methodological work, [2] clarified how Systematic Review (SR)s can identify research gaps, and [21] showed how mapping and synthesis highlight trends

and future directions. This combined guidance informed a structured yet flexible methodology suitable for ageing, AI, and health research.

2.1.1 Search Strategy and Database Screening

The main topic of interest was chronic disease prediction using nutrition, lifestyle, and clinical data from TILDA. Relevant databases (e.g., ScienceDirect, PubMed) were searched using combinations of keywords such as “Irish Longitudinal Study on Ageing”, “machine learning”, “chronic disease”, “nutrition”, “diet”, and “physical activity”.

The search covered studies published from 2011 to 2025. Figure 2.1 summarises the number of studies identified per year, highlighting trends and gaps over time.

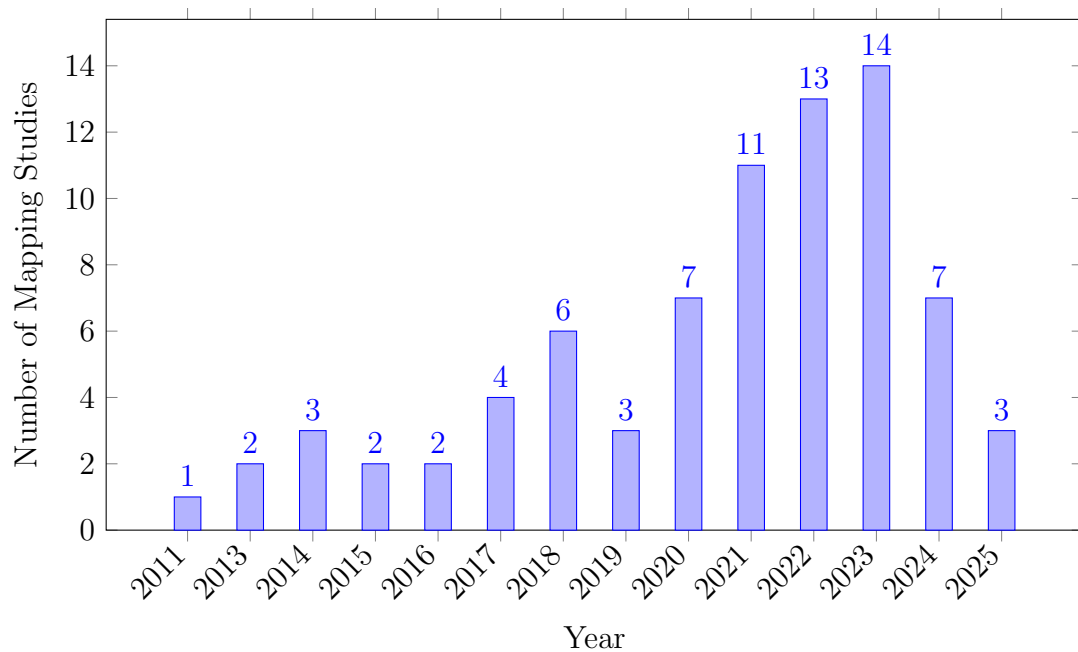


Figure 2.1: Publications per year.

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Table 2.1: Database searches and number of studies per database 2.3 [1].

Database	Search	Results
Oxford Academic	(TILDA OR Irish Longitudinal Study on Ageing)AND (chronic disease OR diabetes OR cardiovascular) AND (nu- trition OR diet OR "physical activity")	1081
	(TILDA OR "Irish Longitudinal Study on Ageing") AND ("machine learning" OR "predictive model" OR "risk pre- diction")	71
IEEE Xplore	TILDA OR "Irish Longitudinal Study on Ageing"	56
	(TILDA OR "Irish Longitudinal Study on Ageing") AND ("machine learning" OR "predictive model" OR "risk pre- diction")	7
ScienceDirect	(TILDA OR "Irish Longitudinal Study on Ageing") AND ("chronic disease" OR cardiovascular OR diabetes) AND (nutrition OR diet OR lifestyle OR "physical activity")	257
	(TILDA OR "Irish Longitudinal Study on Ageing") AND ("machine learning" OR "predictive model") AND ("chronic disease" OR diabetes) AND (nutrition OR "physical activ- ity")	20
PubMed	(TILDA OR "Irish Longitudinal Study on Ageing") AND ("chronic disease" OR cardiovascular OR diabetes) AND (nutrition OR diet OR lifestyle OR "physical activity")	27
	(TILDA OR "Irish Longitudinal Study on Ageing") AND ("machine learning" OR "predictive model" OR "risk pre- diction")	9

Some search strings returned few or irrelevant results, particularly for ML related TILDA studies, highlighting a key research gap. Iterations continued until suitable combinations consistently returned relevant studies. Duplicates were removed using Zotero’s reference management feature. The remaining studies were individually and manually checked for relevance to ‘ageing’, ‘chronic disease’, and ML. Using Zotero’s notes functionality, the initially created table captured key elements for each paper, including model type, features used, and main findings for each paper in the collection, as illustrated in the PRISMA flow diagram (Figure 2.2).

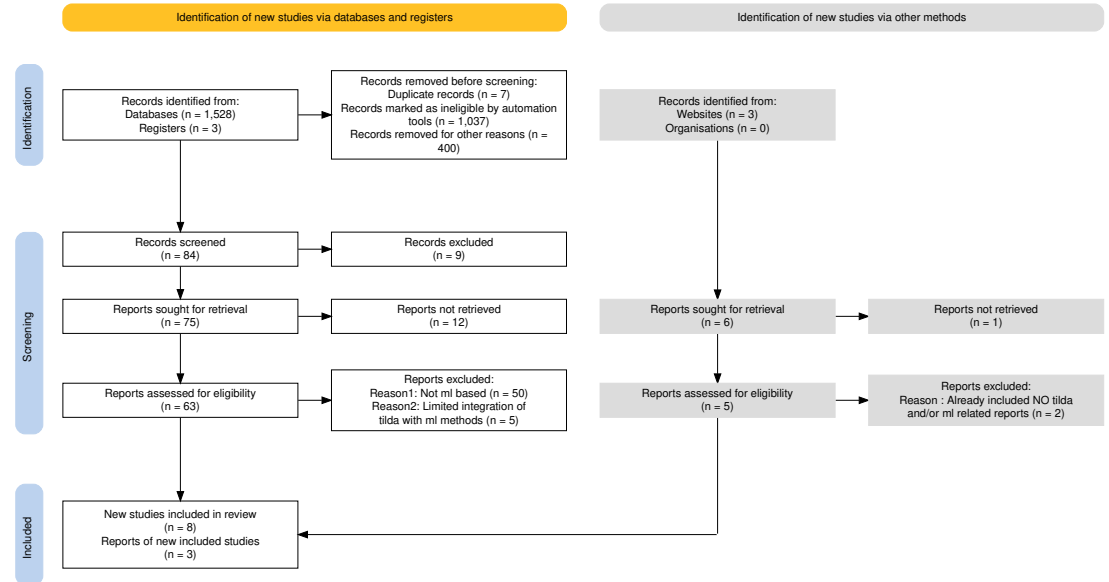


Figure 2.2: PRISMA chart for SRs showing the study selection process for the systematic mapping and integrative review [3].

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2.1.2 Quality and Relevance Evaluation

To systematically assess the quality of the included studies, a customisable version of Newcastle-Ottawa Scale (NOS) was applied, Table 2.2. The NOS evaluates studies across three domains: Selection (S1-S4), Comparability (C1-C2), and Outcome (O1-O3). Selection criteria assess the representativeness of the sample, clarity of exposure measurement, and temporal alignment of outcomes. Comparability examines the control of key confounders, while Outcome evaluates measurement validity, follow up adequacy, and completeness. Each domain is scored 0-1, recall Bernoulli random variable (0 = criterion not met, 1 = criterion met), and the total score reflects the overall study quality.

This initial table guided a further refinement of the results into two sub tables: (1) studies critically related to TILDA, (2) studies using TILDA data with ML for predictive contexts, which revealed minimal to no integration with TILDA.

Table 2.2: Quality evaluation of Systematic Literature Review (SLR)s [2]

Reference	S1	S2	S3	S4	C1	C2	O1	O2	O3	Total
(Xu, 2023 [22])	1	1	1	1	1	1	1	1	1	9
(Donoghue, 2023 [17])	1	1	1	1	1	1	1	0	1	8
(Moguilner, 2021 [16])	1	1	1	1	1	0	1	1	1	8
(Davis, 2021 [6])	1	1	1	1	1	0	1	1	0	7
(Boyle, 2021 [23])	1	1	1	1	1	0	1	0	1	7
(Tzemah-Shahar, 2022 [24])	1	1	1	0	1	0	1	1	1	7
(A. Shirsath, 2024 [25])	1	1	1	1	1	1	1	0	1	8
(Romero-Ortuno, 2021 [26])	1	1	1	1	1	1	1	1	1	9
(Bardon, 2020 [5])	1	1	1	0	1	0	1	1	1	7
(Zhang, 2025 [27])	1	1	1	1	1	1	1	0	1	8
(Sutin, 2023 [28])	1	1	1	1	0	0	1	1	1	7

Table 2.3 summarises selected studies on ageing using TILDA and ML, reflecting key gaps and opportunities identified during systematic mapping. These steps were aligned with the research matrix created at the beginning, ensuring that the refinement of studies and synthesis of findings stayed closely tied to the original research goals.

2.1.3 Synthesis and Integration

Using the NOS scores, I annotated each paper in Zotero with dataset details, key variables, analytical approach, and main findings to ensure a consistent, critical appraisal. For the systematic component, I systematically extracted and synthesised data to identify trends (e.g., Markov models, logistic regression, 5-fold cross validation) and research gaps [2], complemented by DoctoralWriting resources [29, 30], which clarified how to define research questions and develop logical arguments.

The integration of a research matrix with systematic mapping and integrative synthesis ensured that the methodology was well suited to capturing the complexity of ageing, nutrition, chronic disease risk and predictive modelling studies with and without TILDA.

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Table 2.3: Summary of selected studies on ageing using TILDA

Theme	Author/year	Dataset	Variables/features	Methods/model type	Key Findings/outcome	Limitations
Frailty	Romero-Ortuno (2021) [26]	Wave 1-5	Frailty state, Frailty phenotype (FP) components (Exhaustion(EX), Unexplained weight loss(WL), Weakness(WK), Slowness(SL), Low physical activity(LA))	Multi state Markov model, longitudinal 8 year follow up	32% of pre frail to non frail; frailty strongly predicted 31% mortality	Self report bias; observational design
Falls + ML	Donoghue (2023) [17]	Wave 1-3	Gait, mobility, medications, history of falls	Conditional inference forest; 5-fold Cross-Validation (CV); undersampling for imbalance	Accuracy up to 69.7%; for adults aged 50-64.	Area Under the Receiver Operating Characteristic Curve (AUROC) values were ≤ 0.69 and PRAUC ≤ 0.39 indicating low predictive accuracy; no external validation; cross sectional; Class imbalance
Fear of Falling (FOF)	Zarudsky (2014) [31]	Wave 1	Age, sex, fall history	Cross sectional prevalence analysis	23.3% prevalence; higher in women; 70% with FOF had no recent falls in the last 12 months	Self reported; cross sectional
Functional Mobility	Huang (2019) [32]	Wave 1-2	Measured by Timed Up and Go (mobility test) (TUG), grip strength, physical activity, vision, Body Mass Index (BMI), age	Linear regression, stratified analyses	Age modifies effects; Physical Activity (PA) and grip protect mobility; vision benefit only in middle aged (Beta = -0.032, P = 0.002)	Observational; 2 year follow up
Depression	Jacob (2023) [33]	Wave 1-2	Falls, FOF, HADS-A, Center for Epidemiologic Studies Depression Scale (CES-D)	Prospective logistic regression	Falls predicted incident anxiety (Odds Ratio (OR)=1.58) and depression (OR=1.43)	Self reported falls; short follow up; mediation effect partially explored
	Laird (2023) [15]	Wave 1-5	Moderate-to-Vigorous Physical Activity (MVPA), walking, sociodemographics	Longitudinal regression; mixed model	Higher physical activity reduced depressive symptoms; dose response effect	Self reported PA; observational; limited causal inference
Nutrition	Murphy (2023) [11]	Wave 1-5	Plasma lutein, zeaxanthin	Linear/ordinal logistic regression	Higher carotenoids linked to slower frailty progression Lower odds of becoming frail (ORs = 0.57-0.89)	Observational; not predictive of longitudinal muscle changes
	Bardon (2020) [5]	Wave 1-2	Hospitalisation, functional status, social support, sex, falls	Logistic regression; sex stratified analyses	Functional decline, hospitalisation, poor support increased malnutrition risk; sex differences	Observational(short follow up (2 years)); limited generalisability 65 years, self reported
ML	Xu (2023) [22]	Wave 1-3	105 predictors from 40,000+ variables (social, clinical, mental, family)	Stacked ensemble: RF, XRT, GLM, GBM, Deep Neural Network (DNN); Shapley Additive Explanations (SHAP) explainability	AUC=0.854 for 2 year diabetes prediction; top predictors LDL, cirrhosis, sex	No external validation; short follow up; ensemble complexity limits deployment
	Davis (2021) [6]	Wave 3	Age, grip strength, chair stands, BMI, cognitive tests	Histogram gradient boosting regression; SHAP	r ² test: Usual Gait Speed (UGS) ≈ 0.41 , Maximum Gait Speed (MGS) ≈ 0.46 , GSR ≈ 0.21	Modest predictive power; cross sectional; no external validation
	Zhang (2025) [27]	Longitudinal IMAGEN cohort	Psychopathology, personality, cognition, substance use, environment	Regularised logistic regression; diagnostic	Longitudinal AUC: ED=0.71, MDD=0.64, AUD=0.67; shared transdiagnostic predictors	Limited age range; no external replication
	Boyle (2021) [23]	Wave 3	Brain predicted age difference (brainPAD); processing speed; attention; cognitive flexibility; verbal fluency	Linear regression, correlation analyses	Higher brainPAD associated with lower cognitive performance across multiple domains	Cross sectional; no causal inference; limited sample

2.2 Frailty and Physical Function

Frailty is a reflection of higher vulnerability and declining physiological reserve. In TILDA, it is captured through both the frailty phenotype and the frailty index, with gait speed, grip strength, and mobility decline serving as practical predictors. These measures strongly predict hospitalisation and mortality.

2.2.1 Probabilistic Interpretation of Frailty

One can argue that from a probabilistic perspective, the chance of an adverse outcome (such as mortality, Y) given observed features (such as frailty markers, X) as a conditional probability $P(Y | X)$. This allows us to deduce which frailty features are most predictive of adverse outcomes. Features may be independent, dependent, or mutually exclusive.

The presence or absence of metabolic syndrome represents a mutually exclusive event that can inform risk models.

$$P(X_i \cap X_j) = P(X_i)P(X_j)$$

for independent features X_i and X_j , guiding feature selection and reducing redundancy in model inputs. This reflects the likelihood that an individual with certain frailty characteristics will experience a particular outcome.

Evidence suggests that frailty is dynamic, not irreversible. For example, multi state modelling showed that many prefrail adults transition back to non frail, although established frailty strongly predicts mortality [26]. Metabolic syndrome, on the other hand, speeds up the onset of frailty, which suggests a metabolic pathway of vulnerability. [34]. The presence of metabolic syndrome modifies the conditional probability $P(\text{Frailty} | \text{Metabolic Syndrome})$ and may increase the likelihood of adverse outcomes over time. Another detail is that age matters: obesity predicts mobility decline earlier in life, whereas later in life, traits that protect against this, including grip strength, become more significant [32].

These findings partly conflict: frailty appears both reversible and progressive, depending on risk context. This complexity highlights the need for probabilistic models that can update beliefs about risk as new features are observed, as

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formalised by Bayes theorem:

$$P(Y | X) = \frac{P(X | Y)P(Y)}{P(X)}$$

where $P(Y | X)$ is the posterior probability of outcome Y given features X .

2.2.2 Bayesian Updating of Frailty Risk

This enables us to incorporate prior knowledge and observed data, improving the interpretability of feature outcome relationships in ageing. Here, $P(Y | X)$ is the posterior probability = (Likelihood * Prior / Evidence), representing the updated belief about the outcome after observing features. $P(Y)$ is the prior probability, reflecting baseline risk. $P(X | Y)$ is the likelihood, quantifying the probability of observed features given the outcome, and $P(X)$ is the evidence (the overall probability of the features). This framework allows us to update predictions as new data is observed, improving interpretability in feature outcome relationships.

Limitations remain. Causal inference is limited by observational designs, and predictors are frequently examined separately, ignoring interactions. Most frailty models have only been with the TILDA dataset and with this present study focuses on the same TILDA but with an expansion on the harmonised set to explore more longitudinal prediction within this context. These gaps point to the need for multidimensional approaches. Traditional regression models may not fully capture age dependent interactions, suggesting a role for more flexible methods such as machine learning techniques and models.

2.3 Mental Health in Older Adults

Mental health, including depression, anxiety, loneliness, and social isolation, strongly shapes quality of life. Evidence from TILDA shows strong links to both physical and social factors. [35] found that participation in physical, social, and cognitive activities summarised by the Act-Belong-Commit (ABC) indicator was associated with lower depression and anxiety. This suggests social integration and lifestyle protect mental wellbeing. However, [36] reported that self reported

measures may exaggerate such associations..., raising concerns of bias. These two findings conflict: the effect of social engagement is strong when self reported, weaker when objectively assessed.

Objective data strengthen the case for activity. Accelerometer based measures show a dose response link between moderate to vigorous activity and lower depressive symptoms [12]. Yet reverse pathways are also evident: incident falls and functional decline predict later depression, refereed by FOF [33]. Together, these results reveal a bidirectional relationship, mental health both influences and is influenced by physical decline. Most studies rely on broad self report scales such as CES-D or Hospital Anxiety and Depression Scale (HADS), with little biomarker or clinical validation.

This literature suggests mental health in ageing cannot be modelled as a one way effect. It requires longitudinal, multidimensional approaches that can capture mutual pathways, an area where machine learning is well suited especially when combined with probabilistic models that estimate the likelihood and posterior probability of mental health outcomes given multiple interacting features.

For example, we can estimate:

$$P(\text{Depression} \mid \text{Activity, Falls, Social Engagement}),$$

quantifying how these interacting features together influence mental health outcomes. Probabilistic modelling helps determine which features have the strongest effect and which may be redundant for predictive purposes.

2.4 Nutrition and Biological Ageing

Nutrition is increasingly recognised as a modifiable factor in ageing and frailty. Biomarkers like vitamin D, lutein, and zeaxanthin have been linked to physical and cognitive outcomes in TILDA. [11] showed that higher plasma lutein and zeaxanthin dietary carotenoids found in leafy vegetables were linked to slower frailty progression. By contrast, [37] found vitamin D deficiency predicted diabetes and musculoskeletal decline, especially among adults with low sunlight exposure. These studies point to different nutritional mechanisms: antioxidant protection versus metabolic regulation.

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2.4.1 Bayesian Perspective on Nutritional Biomarkers

The effect of nutrition on ageing outcomes can be formalised probabilistically using Bayes theorem:

$$P(\text{Outcome} \mid \text{Nutritional Status}) = \frac{P(\text{Nutritional Status} \mid \text{Outcome})P(\text{Outcome})}{P(\text{Nutritional Status})},$$

where the likelihood function $P(\text{Nutritional Status} \mid \text{Outcome})$ quantifies how observed nutritional biomarkers relate to health states, and the posterior $P(\text{Outcome} \mid \text{Nutritional Status})$ captures updated beliefs about outcomes, including uncertainty in small sample studies. This helps identify which biomarkers are most informative for predicting ageing outcomes which supports feature selection in ML models.

2.4.2 Contextual and Integrative Considerations

Malnutrition also reflects social and functional risks. [5] found that hospitalisation, functional loss, and poor social support increased malnutrition risk, with notable sex differences. This highlights a limitation in much nutritional research: biological markers are studied without integrating social context.

The difference between biological and chronological age is a major point of debate. Biomarker based metrics that better capture these cumulative exposures, like frailty indices or epigenetic clocks, frequently vary from chronological age. McCarthy and Azizi link metabolic syndrome and nutritional deficits to accelerated biological ageing [38, 39]. Although the majority of the evidence is cross sectional and has little ability to capture changes over time, these indicators offer a more precise prediction of vulnerability. Reliability is also decreased in biomarker research with small sample sizes, which can be formalised through the likelihood function $P(\text{Data} \mid \theta)$ and its impact on posterior uncertainty in Bayesian inference.

Taken together, nutrition interacts with biological, metabolic, and social systems. Isolating single nutrients risks oversimplification. Integrative modelling that combines biomarkers, clinical factors, and social determinants supported by machine learning pipelines offers a more promising route. Models that estimate

the joint distribution of features and outcomes can better capture the complex dependencies relevant to biological ageing.

2.5 Machine Learning in Ageing and Chronic Disease

ML is increasingly applied in ageing research because it can capture non linear and multidimensional relationships that traditional regression models, such as logistic regression, often miss [22, 23].

2.5.1 Feature Sets in ML Studies

Studies using ML in ageing research include many types of predictors: clinical measures, sociodemographic factors, lifestyle habits, cognitive tests, and psychosocial variables. A recurring pattern, however, is that models often underline easily measured physical health data, such as comorbidity counts, mobility scores, or vital signs, while more complex but important predictors, including nutrition and social factors, are underrepresented [31]. This imbalance may limit the ability of models to capture the full complexity of ageing, especially considering that mental health and nutrition exert independent and interacting effects on later life outcomes see (here). Models that focus only on readily available clinical data risk reproducing the same biases seen in traditional regression approaches.

2.5.2 ML Models Used

The methodological approaches used in ageing research span both traditional and advanced models. Logistic regression and Markov models remain common baselines due to their interpretability and simplicity, yet they are often unable to capture non linearities or higher order interactions. More recent work has shifted towards ensemble methods such as random forests (RF) and gradient boosting machines (GBM), as well as deep neural networks (DNN), which generally achieve higher predictive accuracy.

2. LITERATURE REVIEW

Similarly, [40] evaluated mortality prediction using a standard logistic regression model with demographic, clinical, and functional predictors, achieving an AUC of 0.78, whereas, ensemble methods in [22] reached 0.854 for diabetes prediction, showing how flexible, non linear models can better capture complex interactions among ageing related predictors.

2.5.3 Performance and Validation

Predictive performance across studies is generally moderate to strong, with AUC values between 0.70 and 0.90 for outcomes like diabetes, gait speed, depression, and mortality. This indicates that models can distinguish reasonably well between individuals who will or will not experience an outcome.

However, there are important limitations. Many studies rely only on internal validation, testing models on the same data used for training. This can inflate performance estimates and reduce confidence in applying the model to new populations. To add, Most studies are cross sectional, so causal or temporal effects cannot be assessed. When combined with the underuse of nutrition and social predictors, these issues indicate that current models may not fully represent the multidimensional nature of ageing and could produce context specific results rather than generalisable predictions [31].

2.5.4 Explainability in ML for Ageing

As ML models become more complex, interpretability emerges as a critical challenge. Explainability tools like SHAP can highlight feature importance, but their use is inconsistent, particularly in deep learning models [6, 22, 41]. These methods improve transparency and can highlight unexpected drivers of outcomes, such as psychosocial variables being stronger predictors than biomedical ones, yet black box concerns remain, especially for deep learning models, where feature interactions are difficult to track. For ageing research, where clinical adoption requires both accuracy and trust, explainability is not optional but essential. Current efforts show promise but remain fragmented, with no consensus on best practice. This gap links directly to the broader methodological limits discussed in the next section on gaps in the literature.

2.6 Gaps in the Literature

First, most studies rely on TILDA or similar cohorts of Irish and predominantly older adults (50+). This limits confidence in how well results transfer to other populations. The present project focuses on TILDA to study ageing in depth but also makes use of the harmonised dataset, which links TILDA with comparable UK and US cohorts. This helps place findings in a broader international context while retaining a core emphasis on older adults.

Second, there is an over reliance on physical health predictors such as comorbidity counts and mobility measures, while nutrition, social, religious or spiritual, and environmental variables are often overlooked. As shown in [11], nutritional biomarkers like lutein and zeaxanthin are strong predictors of frailty, yet these are rarely integrated into broader ML pipelines. By contrast, studies such as [5] highlight the role of social support and functional decline in malnutrition risk. Together, these examples show that excluding nutrition and social factors risks producing models that reinforce the same biomedical biases seen in regression heavy work. From a Bayesian viewpoint, the omission of key features reduces the explanatory power of the model and weakens the posterior probability $P(\text{Outcome} \mid \text{All Features})$ compared to $P(\text{Outcome} \mid \text{Limited Features})$. Including a richer set of features increases the likelihood that the model accurately reflects the true data generating process.

Third, methodological choices remain conservative. Regression models still dominate, while more advanced approaches such as deep neural networks or transformers are rarely explored. For instance, [40] achieved an AUC of 0.78 using logistic regression for mortality prediction in TILDA, whereas [22] reported an AUC of 0.854 with ensemble methods for diabetes incidence. This contrast demonstrates both the dominance of regression and its weaker performance compared to flexible ML models. Where ensemble or brain age approaches have been attempted, validation is almost always internal, with little cross cohort testing. Weak validation and limited feature sets reduce the generalisability of the estimated conditional probabilities and increase the risk of overfitting. Maximum likelihood estimation and Bayesian inference can provide a formal framework for

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assessing model fit and updating predictions as more diverse and representative data become available.

Fourth, many outcomes remain underexplored. Studies on dementia or frailty frequently focus on cognitive or physical decline while leaving aside outcomes critical to older adults, such as pain, sleep quality, and subjective quality of life. For example, [11] used cross sectional plasma biomarkers to study frailty but did not assess sleep or pain, while gait speed models such as [6] similarly neglect subjective quality of life. This shows how reliance on narrow outcomes leaves broader aspects of wellbeing unmeasured.

Fifth, cross sectional bias and data handling weaken reliability. Much nutritional work, such as [11], relies on single time point biomarker data, which cannot capture longitudinal change. By contrast, studies applying multi wave TILDA data [26] demonstrate the added value of modelling temporal dynamics. Similarly, missing data are often handled through list wise deletion, as in [40], which risks biasing results toward healthier participants, whereas more robust imputation approaches remain underused. Missing or poorly handled data can distort the likelihood function and hence the posterior predictions, reducing confidence in the model’s outputs.

Finally, interpretability challenges persist. While tools like SHAP have been applied to highlight feature importance [22, 41], their use is inconsistent, and consensus on best practice has yet to emerge. Without transparent explanations, even accurate models may face barriers to clinical acceptance. Taken together, these examples show that ageing research continues to rely on narrow predictors, conservative models, and weak validation. Addressing these gaps requires integrative, diverse, and transparent approaches that link multidimensional predictors with explainable ML.

2.7 Chapter Summary

This chapter reviewed evidence on ageing and chronic disease across three domains, frailty and physical function, mental health, and nutrition and biological ageing. We also examined how machine learning has been applied to predict later life outcomes. The findings showed that while frailty is partly reversible, it

remains strongly tied to mortality; that mental health both shapes and is shaped by physical decline; and that nutrition interacts with biological, metabolic, and social systems in ways that are often underestimated.

While machine learning has improved prediction accuracy in ageing research, challenges remain in integrating diverse predictors and ensuring interpretability and longitudinal validation.

This study builds on previous research by leveraging TILDA alongside the harmonised dataset linking UK and US cohorts, enabling longitudinal prediction of ageing outcomes. The main contribution lies in integrating nutrition, lifestyle, and clinical predictors, applying temporal modelling, and emphasising interpretability, addressing key limitations identified in prior work. These directions motivate the methodological choices outlined in Chapter 4.

Chapter 3

Technical Approach

3.1 Study Design

TILDA is a prospective cohort study examining the social, economic, and health circumstances of community dwelling adults aged 50 years and older in Ireland. The present study extends this design by incorporating data from five waves of TILDA, as well as the external RAND harmonised dataset, which aligns TILDA with comparable UK and US cohorts.

3.2 Cohort Definition

The analysis included all participants who provided data in at least one of the TILDA waves. This thesis retained the full eligible sample to maximise statistical power for machine learning. Participants were excluded only when they had no usable data after preprocessing (i.e., all features were missing or non informative).

No exclusions were applied based on age, medical conditions, cognitive status, or attendance at the health assessment. Filtering occurred primarily at the *variable level*, where features with excessive missingness, no variation, or non informative structure were removed (see Chapter 4).

3.2.1 Inclusion/Exclusion criteria

- **Participant Inclusion:** All respondents who contributed data in at least one of the TILDA waves were included, regardless of age.
- **Usable Data Requirement:** Participants were excluded only when every available feature was missing or non-informative after preprocessing.
- **Longitudinal Retention:** Individuals were retained even if they participated in only a subset of waves, enabling maximum dataset utilisation.
- **No Health or Cognitive Exclusions:** No participant was removed on the basis of medical diagnoses, cognitive scores, mobility limitations, or non-attendance at the health centre assessment.

The filtering process was therefore driven by data integrity rather than clinical eligibility. Figure 3.1.

3.3 Data Collection

Data were collected across five waves:

- **Wave 1:** 2009-2011
- **Wave 2:** 2012-2013
- **Wave 3:** 2014-2015
- **Wave 4:** 2016-2018
- **Wave 5:** 2018-2020

Note: The Harmonised dataset was obtained from the Research and Development Corporation (RAND) TILDA repository and is designed to align TILDA with comparable surveys such as RAND glsHRS, ELSA, and SHARE. At present, the harmonised version is limited to Waves 1 and 2 and includes variables from the Demographics, Family, and Health sections. This dataset contains cleaned

3. TECHNICAL APPROACH

and processed variables with consistent naming conventions and model based imputations where applicable. It is included here for reference and comparability with other longitudinal aging studies, but it is *not* the output of the preprocessing and cleaning pipeline applied to the full five wave dataset used in this thesis for machine learning analyses.

Each wave included three components:

1. **Computer Assisted Personal Interview (CAPI):** Home based, computer assisted personal interviews
2. **Self Completion Questionnaire (SCQ):** A paper based self completion survey focused on mental health, lifestyle, and subjective well being
3. **Health Assessment:** Clinical and biomarker measurements in centers or at home

These criteria ensured that participants could provide reliable data for longitudinal analyses.

3.3.1 Raw Dataset Dimensions and Manual Filtering

Before any processing, all raw TILDA datasets were inspected to understand dimensionality and structure. Table 3.1 summarises the number of participants and variables per wave. Initial inspection involved:

- inspecting datatypes for every column,
- quantifying missingness, sentinel values, and unique entries,
- generating descriptive statistics (mean, median, min, max),
- and visuals (histograms, distributions) to identify anomalies.

3.4 Feature Structuring and Thematic Grouping

Table 3.1: Dimensions of Raw TILDA and Harmonised Datasets (Pre-Filtering)

Dataset	Participants (N)	Raw Variables (N)
Wave 1	8501	1619
Wave 2	7206	724
Wave 3	6397	1029
Wave 4	5713	990
Wave 5	4978	983
Harmonised	8501	572

3.3.2 Variable Filtering Criteria

The manual variable filtering process involved applying a set of predefined thresholds to remove non-informative or structurally invalid variables prior to automated preprocessing. These criteria included thresholds for missingness, uniqueness, dominant category frequency, datatype quality, and administrative identifiers (see mapping table in Appendix ??).

3.4 Feature Structuring and Thematic Grouping

To manage dimensionality and facilitate interpretability, all feature (variables) were grouped into six thematic domains grounded in ageing and health literature. Table 3.2 provides a summary.

Inspection of raw data revealed heterogeneous variable types, missing values, and wave-specific inconsistencies. This motivated the structured preprocessing, feature engineering, and longitudinal alignment strategies described in Chapter 4. The raw and harmonised dataset overview, participant numbers, and variable counts provide context for the preprocessing and cleaning steps that follow.

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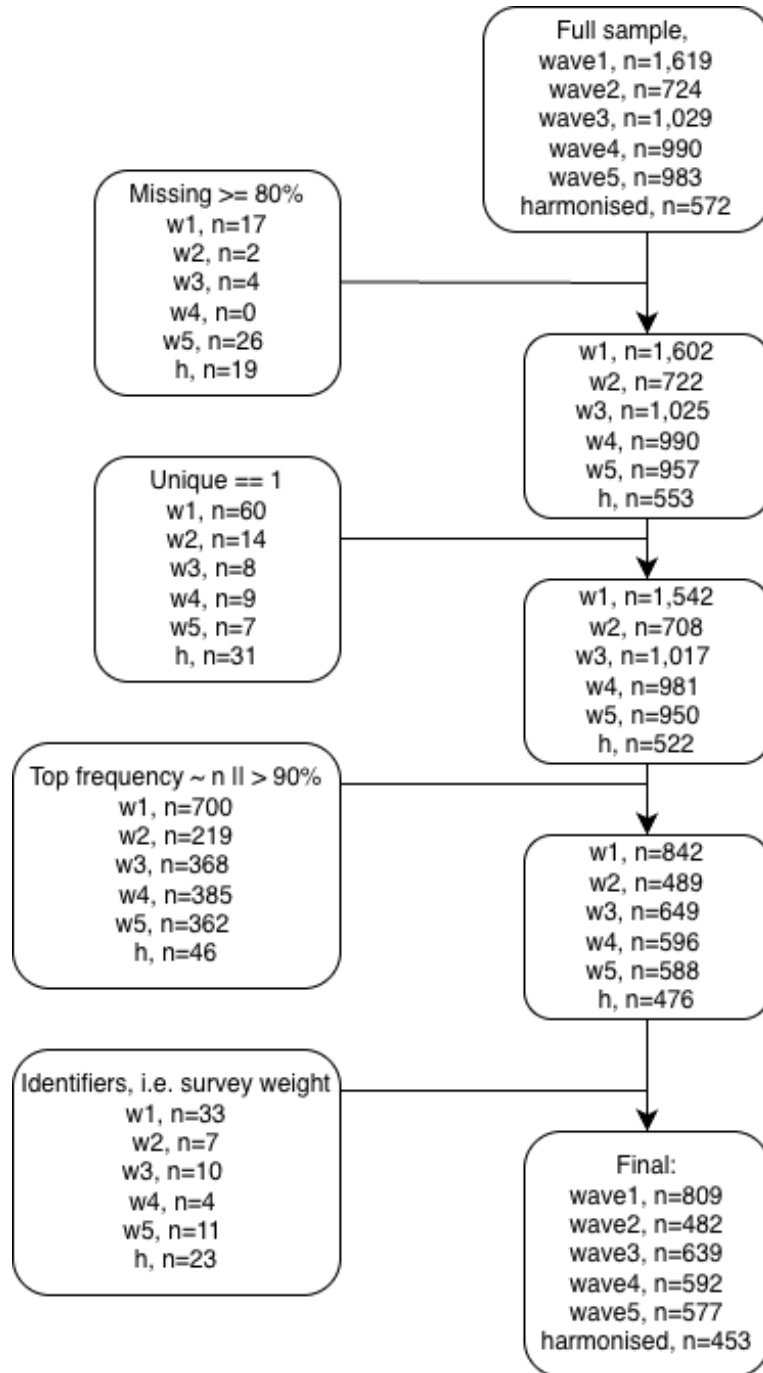


Figure 3.1: Flowchart of manual filtering applied to raw variables for each TILDA wave. Placeholder counts indicate the number of variables remaining after each filtering step.

3.4 Feature Structuring and Thematic Grouping

Table 3.2: Thematic Grouping of Longitudinal Features

ID	Thematic Group	Description	Representative Variables
1	Socio Demographic & Context	Baseline stratification factors	Age, Gender, Education, Income Income Variables (INC), Marital Status Social Variables (SOC)
2	Physical Health & Clinical Baseline	Self-reported and clinical indicators	Self-Rated Health Screening Tests (SCR), BMI, Blood Pressure Blood Pressure from Sphygmomanometer (BP), Comorbidity Count Medication Data (MD)
3	Functional Status & Frailty	Measures of mobility and physical decline	Frailty Index Frailty, Gait, Exercise, Falls and Fractures (FR), Gait Speed, Falls History <i>SCQFalls</i> , Activities of Daily Living (ADL)/IADL Disability, Functional Impairment and Helpers (DIS)
4	Lifestyle & Behavioural	Modifiable health related behaviours	Smoking, Alcohol Intake <i>SCQAlco</i> , Physical Activity <i>IPAQ</i>
5	Nutrition & Metabolic Biomarkers	Objective nutritional and metabolic markers	Plasma Carotenoids, Lutein, Zeaxanthin, Vitamin D/B12, Cholesterol
6	Psychological & Cognitive	Mental health and cognitive performance	CES-D, Loneliness <i>MHucla</i> , Scales and Questions on Cognition (COG), Choice Reaction Time (Cognitive Test) (CRT)

Chapter 4

Implementation Details

4.1 Data Preprocessing

The primary aim of preprocessing was to convert all features, regardless of their original datatype (numeric, categorical, text based, or mixed formats), into a standardised numerical representation suitable for machine learning. All non response and sentinel values were consistently mapped to `-1.0`, and all valid values were converted to `float64`. This ensured compatibility across the five TILDA waves and enabled uniform downstream feature engineering and modelling.

4.1.1 Data Overview

Table 4.1: Dimensions (Rows and Features) of Raw and Harmonised Datasets

Dataset	Row (participants)	Column (variables)
Wave 1	8501	809
Wave 2	7206	482
Wave 3	6397	639
Wave 4	5713	592
Wave 5	4978	577
Harmonised	8501	453

4.1.2 Cleaning Engine

The preprocessing strategy was built around a systematic classification of raw variable types. An exploratory audit across the five waves revealed nine distinct data scenarios (summarised in Appendix 1), ranging from simple numeric fields to complex multi-class string hybrids. The challenge was not merely to clean variables, but to construct a reproducible pipeline that produced *numerical homogeneity* across all waves.

To accomplish this, the nine scenarios were organised into a *Six-Step Cleaning Engine*. The steps reflect a principle of cumulative simplification: resolving simple and unambiguous cases first increases the proportion of clean numerical variables before addressing more complex structures.

- **Steps 1–2: Missingness and sentinel standardisation.** These steps ensured consistent encoding of `-1.0` across all waves.
- **Step 3: Numeric recovery.** Units, symbols, and censored numeric strings (e.g., “50+”) were stripped and converted.
- **Steps 4–6: Categorical complexity.** Ordinal, binary, and multi-class textual variables were processed in order of transformation confidence, with Step 6 applying the most general parsing rules.

This structured sequence ensured that each wave passed through the same deterministic cleaning logic. Figure 4.1 shows the flow of the pipeline.

1. **Empty / Blank Values:** Converts all NaN, empty strings, and whitespace to `-1.0`.
2. **Sentinel Values:** Standardises wave specific non-response codes (e.g., `-98`, `-99`) to `-1.0`.
3. **Numeric Transformation:** Removes non-numeric characters and converts censored or annotated values to numerical equivalents.
4. **Censored Categorical:** Applies known ordinal transformations (e.g., health scales, Likert ranges).

4. IMPLEMENTATION DETAILS

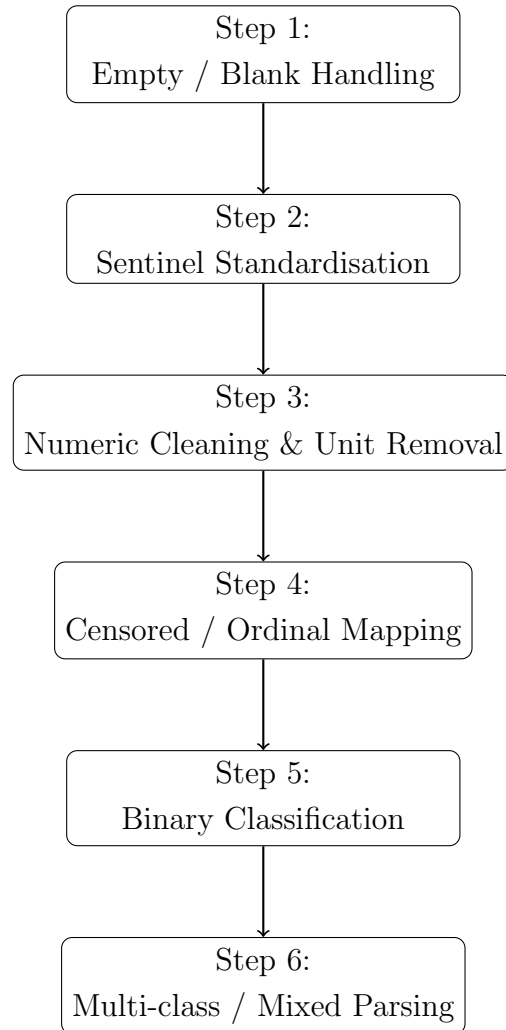


Figure 4.1: Flowchart of the Six-Step Cleaning Pipeline applied to all TILDA waves. See count tables in Appendix 1 for detailed scenario progression at each step.

5. **Binary Categorical:** Automatically maps two level variables (e.g., Yes/No, Male/Female) to 0.0/1.0.
6. **Multi-Class / Mixed:** Applies text parsing, numerical extraction, and rule based mappings to remaining multi-class variables.

4.2 Data Integration and Longitudinal Alignment

Following cleaning, features were aligned across waves based on:

4.3 Summary

This chapter detailed the preprocessing pipeline applied to the five TILDA waves. The approach centred on a reproducible Six-Step Cleaning Engine, combined with systematic wave-to-wave standardisation and thematic feature organisation. These steps produced a structured dataset suitable for longitudinal analysis and machine learning modelling. No cross-cohort harmonisation was performed; any references to a “Harmonised” dataset refer solely to the external RAND Harmonised TILDA.

Chapter 5

Empirical Evaluation

5.1 References

Open the file ‘references_myphd’ with BibTex. The file is located in ‘/7_references/’

The file contains a series of templates for the proper formatting of references according to the Harvard referencing style. Click on each reference in the text below to see how Latex has formatted the reference on the References section.

- **Book:** (? , p. 10) (page number required only if referring to an in-text quote)
- **Book Chapter:** (?)
- **Book Contribution:** (?)
- **MUSIC CD/DVD:** (?)
- **Journal Article:** (?)
- **Patent:** (?)
- **PhD/Master Dissertation:** (?)
- **Conference Paper:** (?)
- **Technical Report:** (?)

- **Unpublished Report:** (?)
- **Web Videos**(?) (. . . not the name of the person who uploaded the video though)
- **Web Paper:** (?)
- **Web Journal Article:** (?)
- **Webpage:** (?)
- **Downloaded Images or Sounds:** (?)

You can also use this kind of referencing if needed in the text:
Surname [?] said that . . .

Chapter 6

Conclusion

6.1 Summary

6.2 Conclusions

6.2.1 Data availability Statement

The data analysed in this study is subject to the following licenses/ restrictions:
The datasets generated during and/or analysed during the current study are not publicly available due to data protection regulations but are accessible at TILDA on reasonable request. Requests to access these datasets should be directed to <https://tilda.tcd.ie/data/accessing-data/>.

6.2.2 Ethics Statement

The studies involving human participants were reviewed and approved by Faculty of Health Sciences Research Ethics Committee at Trinity College Dublin, Ireland. The patients/ participants provided their written informed consent to participate in this study.

6.3 Contributions

6.4 Future Work

Appendix A

Scenario Cleaning Pipeline Tables

Table 1: Initial Raw Feature Counts Categorised by Nine Scenarios (Pre Cleaning)

Scenario	Wave 1	Wave 2	Wave 3	Wave 4	Wave 5	Harmonised
Empty / blank values	9	0	7	0	9	0
Numeric with sentinel	187	59	143	60	100	141
Clean numeric	14	27	18	22	20	8
Censored / bracketed numeric	84	71	91	85	85	73
Binary categorical	248	159	177	220	207	187
Multi class categorical / mixed	267	166	203	205	156	44
Single value categorical	0	0	0	0	0	0
Special symbol / mixed	0	0	0	0	0	0
Unknown / complex mixed	0	0	0	0	0	0
Total Columns	809	482	639	592	577	453

Table 2: Scenario Count Progression: Post Step 1 (Empty / Blank Handling)

Scenario	Wave 1	Wave 2	Wave 3	Wave 4	Wave 5	Harmonised
Empty / blank values	0	0	0	0	0	0
Numeric with sentinel	81	31	42	32	75	15
Clean numeric	124	55	123	50	54	134
Censored / bracketed numeric	84	71	91	85	85	73
Binary categorical	251	159	179	220	207	187
Multi class categorical / mixed	269	166	204	205	156	44
Single value categorical	0	0	0	0	0	0
Special symbol / mixed	0	0	0	0	0	0
Unknown / complex mixed	0	0	0	0	0	0
Total	809	482	639	592	577	453

Table 3: Scenario Count Progression: Post Step 2 (Sentinel Standardisation)

Scenario	Wave 1	Wave 2	Wave 3	Wave 4	Wave 5	Harmonised
Empty / blank values	0	0	0	0	0	0
Numeric with sentinel	0	0	0	0	0	0
Clean numeric	205	86	165	82	129	149
Censored / bracketed numeric	84	71	91	85	85	73
Binary categorical	251	159	179	220	207	187
Multi class categorical / mixed	269	166	204	205	156	44
Single value categorical	0	0	0	0	0	0
Special symbol / mixed	0	0	0	0	0	0
Unknown / complex mixed	0	0	0	0	0	0
Total	809	482	639	592	577	453

Table 4: Scenario Count Progression: Post Step 3 (Numeric Cleaning & Unit Removal)

Scenario	Wave 1	Wave 2	Wave 3	Wave 4	Wave 5	Harmonised
Empty / blank values	0	0	0	0	0	0
Numeric with sentinel	0	0	0	0	0	0
Clean numeric	263	117	215	124	172	149
Censored / bracketed numeric	16	23	17	16	17	73
Binary categorical	251	160	180	221	207	187
Multi class categorical / mixed	279	182	227	231	181	44
Single value categorical	0	0	0	0	0	0
Special symbol / mixed	0	0	0	0	0	0
Unknown / complex mixed	0	0	0	0	0	0
Total	809	482	639	592	577	453

Table 5: Scenario Count Progression: Post Step 4 (Censored / Ordinal Mapping)

Scenario	Wave 1	Wave 2	Wave 3	Wave 4	Wave 5	Harmonised
Empty / blank values	0	0	0	0	0	0
Numeric with sentinel	0	0	0	0	0	0
Clean numeric	279	140	233	140	189	222
Censored / bracketed numeric	0	0	0	0	0	0
Binary categorical	251	160	180	221	207	187
Multi class categorical / mixed	279	182	226	231	181	44
Single value categorical	0	0	0	0	0	0
Special symbol / mixed	0	0	0	0	0	0
Unknown / complex mixed	0	0	0	0	0	0
Total	809	482	639	592	577	453

Appendix

Table 6: Scenario Count Progression: Post Step 5 (Binary Classification)

Scenario	Wave 1	Wave 2	Wave 3	Wave 4	Wave 5	Harmonised
Empty / blank values	0	0	0	0	0	0
Numeric with sentinel	0	0	0	0	0	0
Clean numeric	530	300	413	361	396	409
Censored / bracketed numeric	0	0	0	0	0	0
Binary categorical	0	0	0	0	0	0
Multi class categorical / mixed	279	182	226	231	181	44
Single value categorical	0	0	0	0	0	0
Special symbol / mixed	0	0	0	0	0	0
Unknown / complex mixed	0	0	0	0	0	0
Total	809	482	639	592	577	453

Table 7: Final Dfs Cleaned: Post Step 6 (Multi-Class / Mixed Parsing)

Scenario	Wave 1	Wave 2	Wave 3	Wave 4	Wave 5	Harmonised
Empty / blank values	0	0	0	0	0	0
Numeric with sentinel	0	0	0	0	0	0
Clean numeric	809	482	639	592	577	453
Censored / bracketed numeric	0	0	0	0	0	0
Binary categorical	0	0	0	0	0	0
Multi class categorical / mixed	0	0	0	0	0	0
Single value categorical	0	0	0	0	0	0
Special symbol / mixed	0	0	0	0	0	0
Unknown / complex mixed	0	0	0	0	0	0
Total	809	482	639	592	577	453

Appendix B

Title

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Appendix C

Code for estimating attitude

AHRS_TRIAD.C

```
/*
 *
 * Copyright (c) 2013, Giuseppe Torre
 * All rights reserved.
 *
 * Redistribution and use in source and binary forms, with or without
 * modification, are permitted provided that the following conditions are met:
 *     * Redistributions of source code must retain the above copyright
 *       notice, this list of conditions and the following disclaimer.
 *     * Redistributions in binary form must reproduce the above copyright
 *       notice, this list of conditions and the following disclaimer in the
 *       documentation and/or other materials provided with the distribution.
 *     * Neither the name of the BREAKFAST LLC nor the
 *       names of its contributors may be used to endorse or promote products
 *       derived from this software without specific prior written permission.
 *
 * THIS SOFTWARE IS PROVIDED BY THE COPYRIGHT HOLDERS AND CONTRIBUTORS "AS IS" AND
 * ANY EXPRESS OR IMPLIED WARRANTIES, INCLUDING, BUT NOT LIMITED TO, THE IMPLIED
 * WARRANTIES OF MERCHANTABILITY AND FITNESS FOR A PARTICULAR PURPOSE ARE
 * DISCLAIMED. IN NO EVENT SHALL BREAKFAST LLC BE LIABLE FOR ANY
 * DIRECT, INDIRECT, INCIDENTAL, SPECIAL, EXEMPLARY, OR CONSEQUENTIAL DAMAGES
 * (INCLUDING, BUT NOT LIMITED TO, PROCUREMENT OF SUBSTITUTE GOODS OR SERVICES;
 * LOSS OF USE, DATA, OR PROFITS; OR BUSINESS INTERRUPTION) HOWEVER CAUSED AND
 * ON ANY THEORY OF LIABILITY, WHETHER IN CONTRACT, STRICT LIABILITY, OR TORT
 * (INCLUDING NEGLIGENCE OR OTHERWISE) ARISING IN ANY WAY OUT OF THE USE OF THIS
 * SOFTWARE, EVEN IF ADVISED OF THE POSSIBILITY OF SUCH DAMAGE.
 */

#include "ext.h"
#include "ext_mess.h"
#include <string.h>
#include <stdio.h>
#include <math.h>
#define EPSILON 1e-6

//*****CALIBRATION STUFF*****//
/* INTRODUCE THE OFFSET VALUES IN THE FOLLOWING VARIABLES IN ADC values */
#define ACCELX_OFFSET 2043 //initial offset in accelerometer X
#define ACCELY_OFFSET 2053 // initial offset in accelerometer Y
#define ACCELZ_OFFSET 2264//initial offset in accelerometer Z

#define MagX_OFFSET 1917 //initial offset in Magnetometer X
```



```

#define MagY_OFFSET 1813 // initial offset in Magnetometer Y
#define MagZ_OFFSET 1667 // initial offset in Magnetometer Z
/* WE SHOULD CALIBRATE THE FOLLOWING VALUES FOR EACH PARTICULAR IMU axis */
#define ACCELX_Resolution 536
#define ACCELY_Resolution 661
#define ACCELZ_Resolution 559
#define MagX_Resolution 186
#define MagY_Resolution 189
#define MagZ_Resolution 170
// ***** END CALIBRATION STUFF *****

void *this_class; // Required. Global pointing to this class

typedef struct _triad // Data structure for this object
{
    t_object m_ob; // Must always be the first field; used by Max

    Atom m_args[9]; // we want our inlet to be receiving a list of 10 elements

    long m_value; //inlet

    void *m_R1; //these are all the outlets for the 3 X 3 Matrix
    void *m_R2; // R1 -> first top left cell ..R2 middle cell of the first
    row ...and so on
    void *m_R3; //
    void *m_R4; // End Outlets
} t_triad;

void *triad_new(long value);

void triad_assist(t_triad *triad, void *b, long msg, long arg, char *
s);
void triad_free(t_triad *triad);

void triad_list(t_triad *x, Symbol *s, short argc, t_atom *argv);

void MatrixByMatrix(double *Result, double *MatrixLeft, double *
MatrixRight);

void Matrix2Quat(double *Quat, double *Matrix);
void Quat2Matrix(double *Matrix, double *Quat);
void inverseQuat(double *InvQuat, double *RegQuat);
void NormQuat(double *YesQuat, double *NotQuat);
void Slerp(double *NewQuat, double *OldQuat, double *CurrentQuat);
void NormVect(double *YesVect, double *NotVect);
double orientationMatrix[9];
double Result[9];
int i;
double InorientationMatrix[9];

double temp[6];
double ref[6];
double vectAx[3];
double vectAy[3];
double vectAz[3];
double vectBx[3];
double vectBy[3];
double vectBz[3];
double MagnCrosProd_A;
double MagnCrosProd_B;
double accnorm, magnorm, earthnorm, VectAynorm, VectAznorm,
VectBynorm, VectBznorm;

```

Appendix

```
double m[9], n[9];
double quat_e[4];
double invquat_e[4];
double mult;
double quat_new[4];
double quat_old[4];
double ecs, accex_ADCnumber, y, accey_ADCnumber, z, accez_ADCnumber, mx,
magnx_ADCnumber, my, magny_ADCnumber, mz, magnz_ADCnumber;

// SLERP Variables

double trace, Suca;
double tol[4], omega, sinom, cosom, scale0, scale1, tez,
orientationMatrixA[9];

int main(void)
{
    // set up our class: create a class definition
    setup((t_messlist**) &this_class, (method)triad_new, (method)triad_free, (short)
sizeof(t_triad), 0L, A_GIMME, 0);
    address((method)triad_list, "list", A_GIMME, 0);
    address((method)triad_assist, "assist", A_CANT, 0);
    finder_addclass("Maths", "triad");
    post(".... I'm TRIAD-Object !.... from AHRS-Library ...", 0);
    return 0;
}

/* ----- triad_new ----- */

void *triad_new(long value)
{
    t_triad *triad;
    triad = (t_triad *)newobject(this_class); // create the new instance and return
a pointer to it
    triad->m_R4 = floatout(triad);
    triad->m_R3 = floatout(triad);
    triad->m_R2 = floatout(triad);
    triad->m_R1 = floatout(triad);

    return(triad);
}

etc.

etc.
```

References

- [1] Kai Petersen, Sairam Vakkalanka, and Ludwik Kuzniarz. Guidelines for conducting systematic mapping studies in software engineering: An update. *Information and Software Technology*, 64:1–18, August 2015. ISSN 0950-5849. doi: 10.1016/j.infsof.2015.03.007. vi, 6, 8
- [2] Barbara Kitchenham, O. Pearl Brereton, David Budgen, Mark Turner, John Bailey, and Stephen Linkman. Systematic literature reviews in software engineering – A systematic literature review. *Information and Software Technology*, 51(1):7–15, January 2009. ISSN 09505849. doi: 10.1016/j.infsof.2008.09.009. vi, 6, 10, 11
- [3] Neal Haddaway, Matthew J Page, Chris C Pritchard, and Luke A McGuinness. PRISMA2020: An R package and Shiny app for producing PRISMA 2020-compliant flow diagrams, with interactivity for optimised digital transparency and Open Synthesis, 2022. vii, 9
- [4] Leonardo Bencivenga, Klara Komici, Pierangela Nocella, Fabrizio Vincenzo Grieco, Angela Spezzano, Brunella Puzone, Alessandro Cannavo, Antonio Cittadini, Graziamaria Corbi, Nicola Ferrara, and Giuseppe Rengo. Atrial fibrillation in the elderly: A risk factor beyond stroke. *Ageing Research Reviews*, 61:101092, 2020. ISSN 1568-1637. doi: 10.1016/j.arr.2020.101092. 1
- [5] Laura A Bardon, Melanie Streicher, Clare A Corish, Michelle Clarke, Lauren C Power, Rose Anne Kenny, Deirdre M O’Connor, Eamon Laird, Eibhlis M O’Connor, Marjolein Visser, Dorothee Volkert, Eileen R Gibney, and MaNuEL Consortium. Predictors of Incident Malnutrition in Older Irish Adults from the Irish Longitudinal Study on Ageing Cohort—A MaNuEL study. *J Gerontol A Biol Sci Med Sci*, 75(2):249–256, January 2020. ISSN 1079-5006. doi: 10.1093/gerona/gly225. 1, 10, 12, 16, 19

REFERENCES

- [6] James R. C. Davis, Silvin P. Knight, Orna A. Donoghue, Belinda Hernández, Rossella Rizzo, Rose Anne Kenny, and Roman Romero-Ortuno. Comparison of Gait Speed Reserve, Usual Gait Speed, and Maximum Gait Speed of Adults Aged 50+ in Ireland Using Explainable Machine Learning. *Front Netw Physiol*, 1:754477, 2021. ISSN 2674-0109. doi: 10.3389/fnetp.2021.754477. 1, 10, 12, 18, 20
- [7] Patrick V. Moore, Kathleen Bennett, and Charles Normand. Counting the time lived, the time left or illness? Age, proximity to death, morbidity and prescribing expenditures. *Social Science & Medicine*, 184:1–14, 2017. ISSN 0277-9536. doi: 10.1016/j.socscimed.2017.04.038. 1
- [8] Ying Huang, Yuhao Su, Ying Jiang, and Meilan Zhu. Sex differences in the associations between blood pressure and anxiety and depression scores in a middle-aged and elderly population: The Irish Longitudinal Study on Ageing (TILDA). *Journal of Affective Disorders*, 274:118–125, September 2020. ISSN 0165-0327. doi: 10.1016/j.jad.2020.05.133. 1
- [9] World Health Organization Europe. New data: Noncommunicable diseases cause 1.8 million avoidable deaths and cost US\$ 514 billion every year, reveals new WHO/Europe report. <https://www.who.int/europe/news/item/27-06-2025-new-data-noncommunicable-diseases-cause-1-8-million-avoidable-deaths-and-cost-us-514-billion-USD-every-year-reveals-new-who-europe-report>, 2025. 1
- [10] Niamh Rice and Charles Normand. The cost associated with disease-related malnutrition in Ireland. *Public Health Nutr*, 15(10):1966–1972, October 2012. ISSN 1475-2727. doi: 10.1017/S1368980011003624. 1
- [11] Caoileann H. Murphy, Eoin Duggan, James Davis, Aisling M. O’Halloran, Silvin P. Knight, Rose Anne Kenny, Sinead N. McCarthy, and Roman Romero-Ortuno. Plasma lutein and zeaxanthin concentrations associated with musculoskeletal health and incident frailty in The Irish Longitudinal Study on Ageing (TILDA). *Experimental Gerontology*, 171:112013, 2023. ISSN 0531-5565. doi: 10.1016/j.exger.2022.112013. 1, 3, 12, 15, 19, 20
- [12] Eamon Laird, Charlotte Lund Rasmussen, Rose Anne Kenny, and Matthew P. Herring. Physical Activity Dose and Depression in a Cohort of Older Adults in The Irish Longitudinal Study on Ageing. *JAMA Netw Open*, 6(7):e2322489, July 2023. ISSN 2574-3805. doi: 10.1001/jamanetworkopen.2023.22489. 1, 15

REFERENCES

- [13] C. P. McDowell, R. K. Dishman, M. Hallgren, C. MacDonncha, and M. P. Herring. Associations of physical activity and depression: Results from the Irish Longitudinal Study on Ageing. *Experimental Gerontology*, 112:68–75, 2018. ISSN 0531-5565. doi: 10.1016/j.exger.2018.09.004. 1
- [14] Cathal McCrory, Ciaran Finucane, Celia O’Hare, John Frewen, Hugh Nolan, Richard Layte, Patricia M. Kearney, and Rose Anne Kenny. Social Disadvantage and Social Isolation Are Associated With a Higher Resting Heart Rate: Evidence From The Irish Longitudinal Study on Ageing. *J Gerontol B Psychol Sci Soc Sci*, 71(3):463–473, May 2016. ISSN 1079-5014. doi: 10.1093/geronb/gbu163. 2
- [15] Eamon Laird, Matthew P. Herring, Brian P. Carson, Catherine B. Woods, Cathal Walsh, Rose Anne Kenny, and Charlotte Lund Rasmussen. Physical activity for depression among the chronically ill: Results from older diabetics in the Irish longitudinal study on ageing. *Psychiatry Research*, 326:115274, August 2023. ISSN 0165-1781. doi: 10.1016/j.psychres.2023.115274. 3, 12
- [16] Sebastian Moguilner, Silvin P. Knight, James R. C. Davis, Aisling M. O’Halloran, Rose Anne Kenny, and Roman Romero-Ortuno. The Importance of Age in the Prediction of Mortality by a Frailty Index: A Machine Learning Approach in the Irish Longitudinal Study on Ageing. *Geriatrics (Basel)*, 6(3):84, August 2021. ISSN 2308-3417. doi: 10.3390/geriatrics6030084. 3, 10
- [17] Orna A Donoghue, Belinda Hernandez, Matthew D L O’Connell, and Rose Anne Kenny. Using Conditional Inference Forests to Examine Predictive Ability for Future Falls and Syncope in Older Adults: Results from The Irish Longitudinal Study on Ageing. *J Gerontol A Biol Sci Med Sci*, 78(4):673–682, April 2023. ISSN 1758-535X. doi: 10.1093/gerona/glac156. 3, 10, 12
- [18] Richard J. Torraco. Writing Integrative Literature Reviews: Guidelines and Examples. *Human Resource Development Review*, 4(3):356–367, September 2005. ISSN 1534-4843, 1552-6712. doi: 10.1177/1534484305278283. 6
- [19] Tonette S. Rocco and Maria S. Plakhotnik. Literature Reviews, Conceptual Frameworks, and Theoretical Frameworks: Terms, Functions, and Distinctions. *Human Resource Development Review*, 8(1):120–130, March 2009. ISSN 1534-4843, 1552-6712. doi: 10.1177/1534484309332617. 6
- [20] Andrew D Oxman and Gordon H Guyatt. Guidelines for reading literature reviews.

REFERENCES

- [21] Muhammad Azeem Ashraf, Meijia Yang, Yufeng Zhang, Mouna Denden, Ahmed Tlili, Jiayi Liu, Ronghuai Huang, and Daniel Burgos. A Systematic Review of Systematic Reviews on Blended Learning: Trends, Gaps and Future Directions. *Psychology Research and Behavior Management*, 14:1525–1541, October 2021. ISSN null. doi: 10.2147/PRBM.S331741. 6
- [22] Xuezhong Xu, Xue Mingyang, Jie Yang, Hailong Zheng, and Zhifei Che. Tailored machine learning for evaluating the long-term diabetes risk in older individuals: Findings from the Irish Longitudinal Study on Ageing (TILDA). *BMJ Open*, 13(5):e072991, May 2023. ISSN 2044-6055. doi: 10.1136/bmjopen-2023-072991. 10, 12, 17, 18, 19, 20
- [23] Rory Boyle, Lee Jollans, Laura M. Rueda-Delgado, Rossella Rizzo, Görsev G. Yener, Jason P. McMorro, Silvin P. Knight, Daniel Carey, Ian H. Robertson, Derya D. Emek-Savaş, Yaakov Stern, Rose Anne Kenny, and Robert Whelan. Brain-predicted age difference score is related to specific cognitive functions: A multi-site replication analysis. *Brain Imaging Behav*, 15(1):327–345, February 2021. ISSN 1931-7565. doi: 10.1007/s11682-020-00260-3. 10, 12, 17
- [24] Roy Tzemah-Shahar, Hagit Hochner, Khalil Iktilat, and Maayan Agmon. What can we learn from physical capacity about biological age? A systematic review. *Ageing Research Reviews*, 77:101609, May 2022. ISSN 1568-1637. doi: 10.1016/j.arr.2022.101609. 10
- [25] Morgana A. Shirsath, John D. O’Connor, Rory Boyle, Louise Newman, Silvin P. Knight, Belinda Hernandez, Robert Whelan, James F. Meaney, and Rose Anne Kenny. Slower speed of blood pressure recovery after standing is associated with accelerated brain ageing: Evidence from The Irish Longitudinal Study on Ageing (TILDA). *Cerebral Circulation - Cognition and Behavior*, 6:100212, January 2024. ISSN 2666-2450. doi: 10.1016/j.cccb.2024.100212. 10
- [26] Roman Romero-Ortuno, Peter Hartley, James Davis, Silvin P. Knight, Rossella Rizzo, Belinda Hernández, Rose Anne Kenny, and Aisling M. O’Halloran. Transitions in frailty phenotype states and components over 8 years: Evidence from The Irish Longitudinal Study on Ageing. *Archives of Gerontology and Geriatrics*, 95:104401, 2021. ISSN 0167-4943. doi: 10.1016/j.archger.2021.104401. 10, 12, 13, 20
- [27] Zuo Zhang, Lauren Robinson, Robert Whelan, Lee Jollans, Zijian Wang, Frauke Nees, Congying Chu, Marina Bobou, Dongping Du, Ilinca Cristea, Tobias Ba-

REFERENCES

- naschewski, Gareth J. Barker, Arun L. W. Bokde, Antoine Grigis, Hugh Garavan, Andreas Heinz, Rüdiger Brühl, Jean-Luc Martinot, Marie-Laure Paillère Martinot, Eric Artiges, Dimitri Papadopoulos Orfanos, Luise Poustka, Sarah Hohmann, Sabina Millenet, Juliane H. Fröhner, Michael N. Smolka, Nilakshi Vaidya, Henrik Walter, Jeanne Winterer, M. John Broulidakis, Betteke Maria van Noort, Argyris Stringaris, Jani Penttilä, Yvonne Grimmer, Corinna Insensee, Andreas Becker, Yuning Zhang, Sinead King, Julia Sinclair, Gunter Schumann, Ulrike Schmidt, and Sylvane Desrivières. Machine learning models for diagnosis and risk prediction in eating disorders, depression, and alcohol use disorder. *Journal of Affective Disorders*, 379:889–899, June 2025. ISSN 0165-0327. doi: 10.1016/j.jad.2024.12.053. 10, 12
- [28] Dr Angelina R. Sutin, Martina Luchetti, Damaris Aschwanden, Yannick Stephan, Amanda A. Sesker, and Antonio Terracciano. Sense of meaning and purpose in life and risk of incident dementia: New data and meta-analysis. *Archives of Gerontology and Geriatrics*, 105:104847, 2023. ISSN 0167-4943. doi: 10.1016/j.archger.2022.104847. 10
- [29] doctoralwriting. Creating the literature review: Research questions and arguments, February 2016. 11
- [30] doctoralwriting. Creating the literature review: Research questions and arguments (Part 3 of 4), February 2016. 11
- [31] A. A. Zarudsky and K. I. Prashchayeu. P277: One-legged balance test and falls in old patients with cardiovascular diseases. *European Geriatric Medicine*, 5:S171–S172, 2014. ISSN 1878-7649. doi: 10.1016/S1878-7649(14)70448-6. 12, 17, 18
- [32] Chuanying Huang, Shuqin Sun, Xiaocao Tian, Tong Wang, Tianyu Wang, Haiping Duan, and Yili Wu. Age modify the associations of obesity, physical activity, vision and grip strength with functional mobility in Irish aged 50 and older. *Archives of Gerontology and Geriatrics*, 84:103895, 2019. ISSN 0167-4943. doi: 10.1016/j.archger.2019.05.020. 12, 13
- [33] Louis Jacob, Karel Kostev, Jae Il Shin, Lee Smith, Hans Oh, Adel S. Abduljabbar, Josep Maria Haro, and Ai Koyanagi. Falls increase the risk for incident anxiety and depressive symptoms among adults aged ≥ 50 years: An analysis of the Irish longitudinal study on ageing. *Archives of Gerontology and Geriatrics*, 114:105098, 2023. ISSN 0167-4943. doi: 10.1016/j.archger.2023.105098. 12, 15

REFERENCES

- [34] Kevin McCarthy, Eamon Laird, Aisling M. O'Halloran, Padraic Fallon, Román Romero Ortuño, and Rose Anne Kenny. Association between metabolic syndrome and risk of both prevalent and incident frailty in older adults: Findings from The Irish Longitudinal Study on Ageing (TILDA). *Experimental Gerontology*, 172:112056, 2023. ISSN 0531-5565. doi: 10.1016/j.exger.2022.112056. 13
- [35] Ziggi Ivan Santini, Ai Koyanagi, Stefanos Tyrovolas, Josep Maria Haro, Robert J. Donovan, Line Nielsen, and Vibeke Koushede. The protective properties of Act-Belong-Commit indicators against incident depression, anxiety, and cognitive impairment among older Irish adults: Findings from a prospective community-based study. *Experimental Gerontology*, 91:79–87, 2017. ISSN 0531-5565. doi: 10.1016/j.exger.2017.02.074. 14
- [36] A. L. Juola, M. P. Bjorkman, H. Kautiainen, S. Pylkkanen, U. H. Finne-Soveri, H. Soini, K. H. Pitkälä, and J. S. Bell. O1.17: Nursing staff education to reduce potentially harmful medication use among older people in assisted living facilities: Effects of randomized controlled trial on cognition and falls. *European Geriatric Medicine*, 5:S50–S51, 2014. ISSN 1878-7649. doi: 10.1016/S1878-7649(14)70100-7. 14
- [37] Kevin McCarthy, Eamon Laird, Aisling M. O'Halloran, Cathal Walsh, Martin Healy, Annette L. Fitzpatrick, James B. Walsh, Belinda Hernández, Padraic Fallon, Anne M. Molloy, and Rose Anne Kenny. Association between vitamin D deficiency and the risk of prevalent type 2 diabetes and incident prediabetes: A prospective cohort study using data from The Irish Longitudinal Study on Ageing (TILDA). *EClinicalMedicine*, 53:101654, November 2022. ISSN 2589-5370. doi: 10.1016/j.eclinm.2022.101654. 15
- [38] Kevin McCarthy, Aisling M. O'Halloran, Padraic Fallon, Rose Anne Kenny, and Cathal McCrory. Metabolic syndrome accelerates epigenetic ageing in older adults: Findings from The Irish Longitudinal Study on Ageing (TILDA). *Experimental Gerontology*, 183:112314, 2023. ISSN 0531-5565. doi: 10.1016/j.exger.2023.112314. 16
- [39] Zahra Azizi, Rebecca J. Hirst, Alan O' Dowd, Cathal McCrory, Rose Anne Kenny, Fiona N. Newell, and Annalisa Setti. Evidence for an association between allostatic load and multisensory integration in middle-aged and older adults. *Archives of Gerontology and Geriatrics*, 116:105155, 2024. ISSN 0167-4943. doi: 10.1016/j.archger.2023.105155. 16

REFERENCES

- [40] Soraya Matthews, Mark Ward, Anne Nolan, Charles Normand, Rose Anne Kenny, and Peter May. Predicting mortality in The Irish Longitudinal Study on Ageing (TILDA): Development of a four-year index and comparison with international measures. *BMC Geriatr*, 22:510, June 2022. ISSN 1471-2318. doi: 10.1186/s12877-022-03196-z. 18, 19, 20
- [41] James Davis, Silvin P. Knight, Rossella Rizzo, Orna A. Donoghue, Rose Anne Kenny, and Roman Romero-Ortuno. A linear regression-based machine learning pipeline for the discovery of clinically relevant correlates of gait speed reserve from multiple physiological systems. In *2021 29th European Signal Processing Conference (EUSIPCO)*, pages 1266–1270, August 2021. doi: 10.23919/EUSIPCO54536.2021.9616187. 18, 20